

NOTE

Admit: 07.05.2022

Expired: 12.05.2022

Service: Hematology/Oncology → Palliative Care

Attending: [Name], MD

Primary Dx: AML, **ELN 2022 Adverse**, **SRSF2** mutation (myelodysplasia-related), complex clonal evolution over time.

Initial therapy: **Decitabine** initiated **12.05.2021**.

History of Present Illness

Mr. Brian Wright (DOB 18.09.53, MRN AML034), a 68-year-old retired machinist, presented one year earlier with anemia, easy bruising, and neutropenic infections. Bone marrow biopsy in early May 2021 showed 55% blasts with dysplasia; cytogenetics initially normal but later revealed additional aberrations. NGS identified **SRSF2** p.P95H and **ASXL1** co-mutation at low VAF. Given age, frailty (ECOG 2), and ischemic cardiomyopathy, he received low-intensity **decitabine** on a 28-day schedule with transfusion support.

He achieved transient hematologic improvement; by early **February 2022** he developed progressive cytopenias and circulating blasts. A marrow in mid-February showed refractory disease with 38% blasts. After discussion of options, he declined intensive salvage; a trial of **azacitidine + venetoclax** was considered but deferred due to recurrent infections and worsening ejection fraction (LVEF 35%). He opted for comfort-focused care if condition deteriorated.

Terminal Admission Course (07–12 May 2022)

Reason for admission: Progressive dyspnea, hypotension, and fever at home. On arrival: T 38.6°C, BP 86/54 (MAP 62), HR 118, RR 26, SpO₂ 91% on 2 L. Exam notable for petechiae, oral thrush, and crackles at bases.

Key Data on Admission:

- **CBC:** WBC $1.1 \times 10^9/L$ (blasts 12%), ANC 0.05, Hgb 6.9 g/dL, Plt $7 \times 10^9/L$.
- **CMP:** Na 132, K 4.8, Cr 1.9 (baseline 1.1), AST/ALT 68/74, TBili 1.3.
- **Coag:** INR 1.4, fibrinogen 410. **LDH** 820 U/L. **Lactate** 4.3 mmol/L.
- **Imaging:** CXR with bilateral patchy opacities; cardiomegaly.
- **Cultures:** Blood grew **Candida glabrata** (fluconazole-resistant) and later **E. coli** (ESBL) from line; urine negative.

Management:

- Empiric meropenem + vancomycin; micafungin initiated; central line removed. Platelet and RBC transfusions given per thresholds (Plt <10, Hgb <8).
- Oxygen escalated to high-flow nasal cannula; diuresis limited by hypotension.
- Given refractory shock and progressive hypoxemia, he reiterated his wishes for comfort measures only; DNR/DNI documented. Palliative care managed dyspnea and anxiety with low-dose hydromorphone infusion and lorazepam PRN; glycopyrrolate for secretions. Chaplain and family present.

Trajectory: Despite antibiotics/antifungals, he developed **septic shock** with multiorgan failure (rising creatinine to 2.6, AST/ALT peak 146/180). No meaningful hematologic recovery. He died peacefully at **14:12 on 12.05.2022** with his daughter at bedside.

Problem List

- AML with **SRSF2** mutation, refractory to hypomethylating therapy.
 - Profound pancytopenia with transfusion dependence.
 - Septic shock (mixed **Candida glabrata** and ESBL **E. coli** bacteremia).
 - Ischemic cardiomyopathy (EF 35%), chronic kidney disease stage 3.
 - Type 2 diabetes mellitus (A1c 7.2% prior year), hypertension, former smoker.
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Medications at End of Life

- Micafungin, meropenem; hydromorphone infusion (titrated to comfort), lorazepam PRN, acetaminophen PR, ondansetron PRN, bowel regimen. Chronic agents (ACE-i, metoprolol, metformin) held due to instability.
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Family and Aftercare

Next of kin: **Claire Wright** (daughter). Hospice had been consulted earlier as an outpatient; bereavement resources provided. Autopsy declined. Death certificate completed; copy forwarded to PCP.

Attending signature: _____